The Care Gap

Who goes without mental health care

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In the early 2000s, fewer than two in five U.S. adults with depression reported professional help for it in a given year, and fewer than one in five with social phobia did. These disorders shape how people manage a home, keep close relationships, and take part in social life. Yet having a disorder, even one that badly disrupts daily life, does not mean getting help for it.

This article follows that gap. Using a national survey of more than 10,000 adults from 2001–2003, it first shows how common six disorders were and how often they overlapped, then how much they interfered with daily life, and finally how many people reported care. A last section turns to 2024 data on depression to ask whether the gap has closed. It has not.

Key terms

- **The six disorders.** *Depression* (major depressive disorder) means at least two weeks of low mood or lost interest, with other symptoms. *Generalized anxiety* is persistent, hard-to-control worry. *Panic disorder* is repeated, unexpected panic attacks. *Social phobia* (social anxiety disorder) is intense fear of being judged in social situations. *Agoraphobia* is fear and avoidance of places where escape might be hard; here it means agoraphobia without panic disorder. *PTSD* (post-traumatic stress disorder) is lasting distress after a traumatic event.
- **Past-year prevalence.** The share of adults who met the criteria for a disorder at some point in the 12 months before their interview.
- **Weighted estimates.** The survey did not sample everyone with equal chance, so each answer is weighted to make the results represent U.S. adults.
- **95% confidence interval.** The range of values consistent with the data, given that this is a sample. Wider intervals mean less certainty, usually because fewer people answered.
- **Interference score.** How much a disorder got in the way of daily life during its worst period in the past year, from 0 (none) to 10 (very severe).

Common, and rarely alone

About 7% of adults had depression in the past year, and almost as many had social phobia (6.6%). The other four disorders were less common, from 4.0% for generalized anxiety to 1.4% for agoraphobia. Social phobia, often overlooked next to depression, affected about as many people: their confidence intervals overlap.

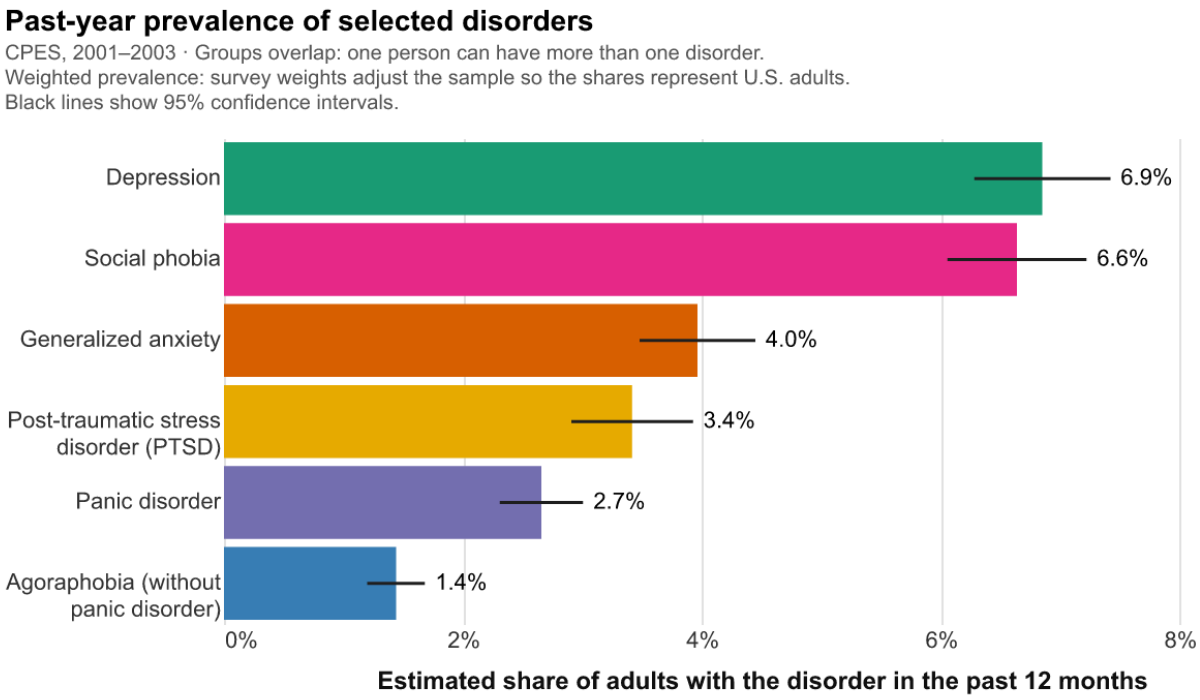


Figure 1. Past-year prevalence of six disorders. Black lines show 95% confidence intervals.

These groups overlap heavily. Among adults with agoraphobia, 63% also had social phobia. More than a third of adults with generalized anxiety also had depression (37%), and about three in ten adults with panic disorder or PTSD also had social phobia.

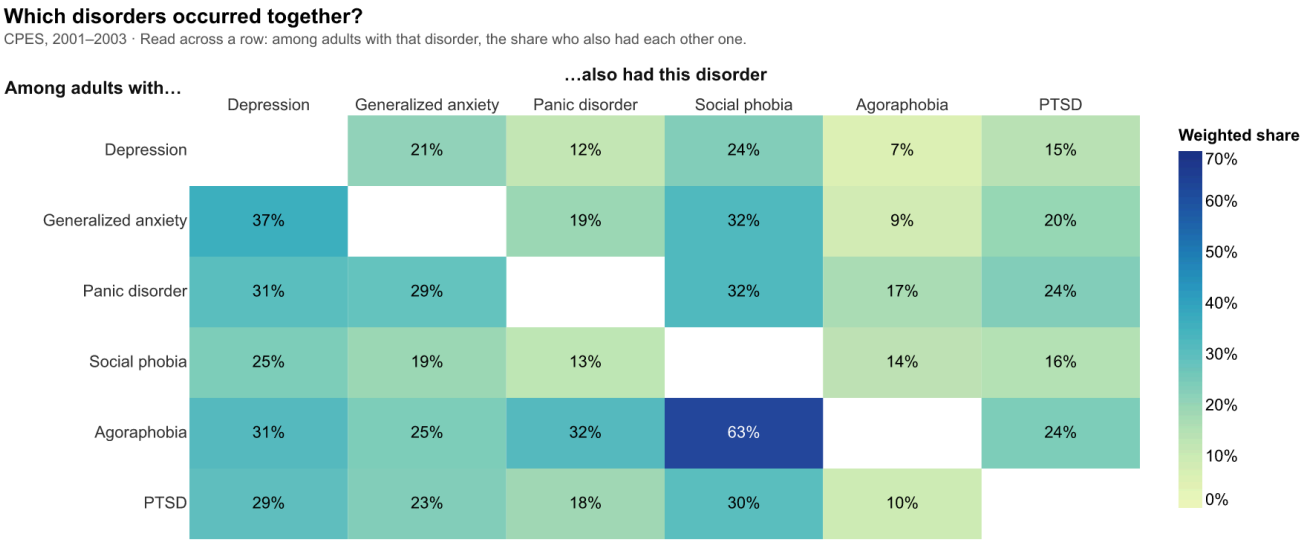


Figure 2. Read across each row: among adults with the row's disorder, the share who also had each other disorder.

Looking at exact combinations makes the pattern clearer. Among the 754 respondents with at least two of the six disorders, the most common pairing was depression with social phobia (14.3%), followed by depression with generalized anxiety (11.7%) and depression with PTSD (7.7%). Depression appears in five of the ten most common combinations, and social phobia in six. Many people were carrying more than one condition at once.

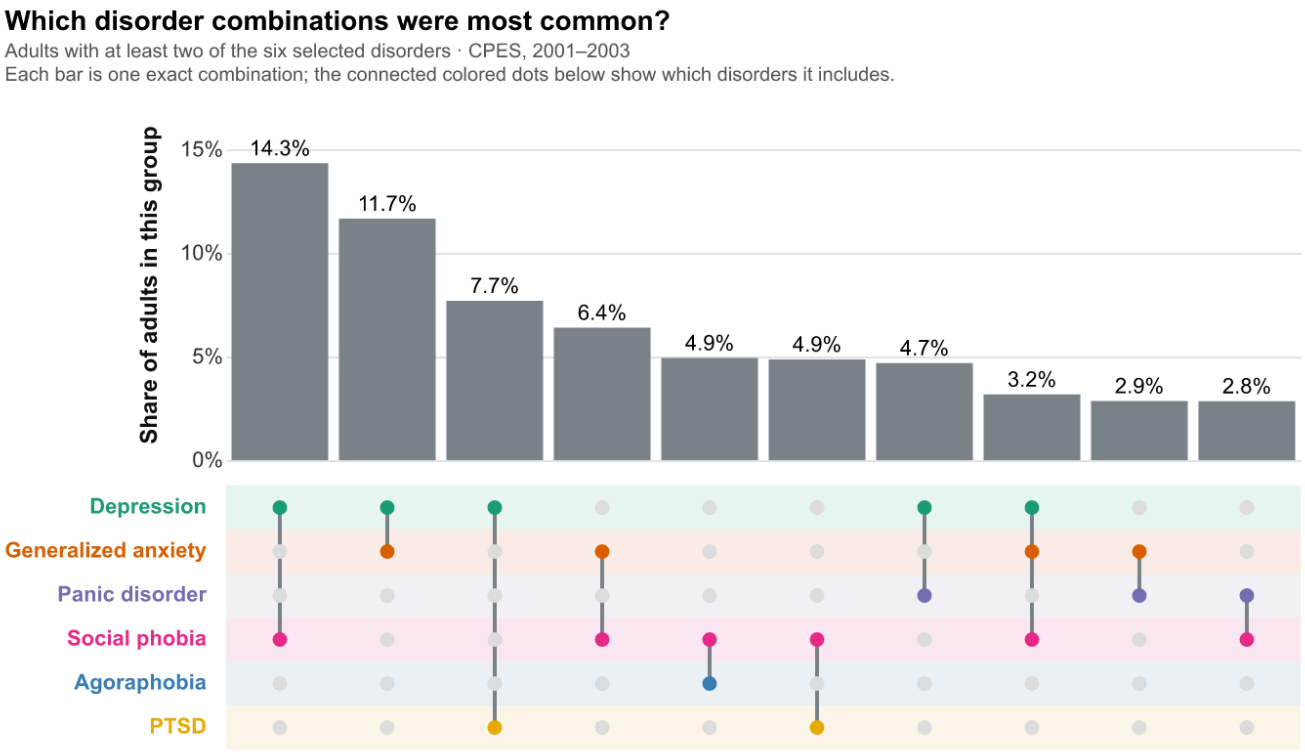


Figure 3. The ten most common exact combinations among adults with at least two of the six disorders; together they account for 63.5% of this group.

What these disorders do to daily life

The survey asked how much each disorder interfered with home management, close relationships, and social life. For five of the six disorders, social life was the most affected of the three. The gap was widest for social phobia: an average of 5.1 for social life, but only 2.5 for home management. For PTSD, close relationships (5.3) edged out social life (5.2).

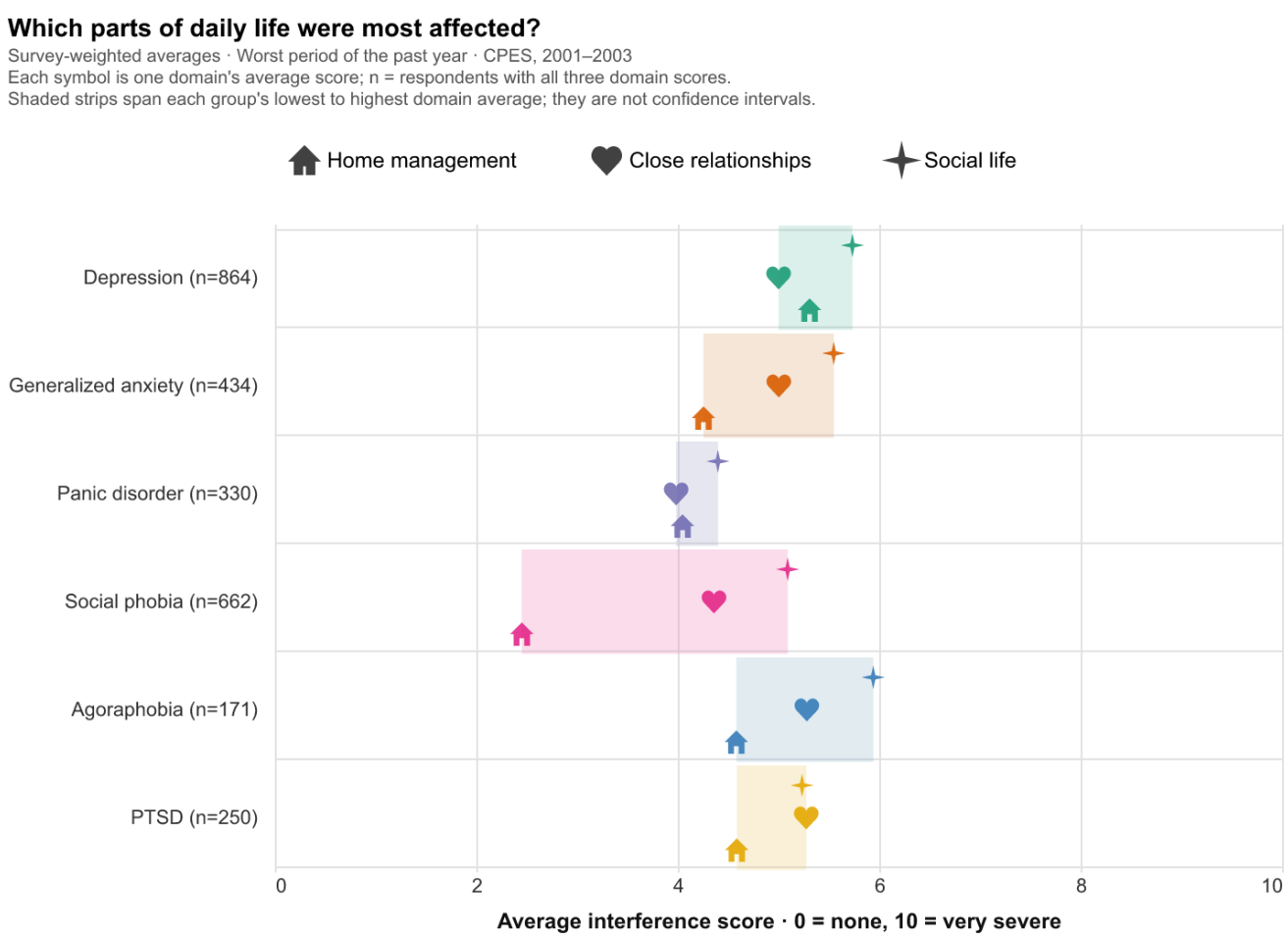


Figure 4. Average interference in three areas of daily life. Shaded strips span each group's lowest to highest average; they are not confidence intervals.

Averages around 4 to 6 can make interference look moderate. The full distribution of social-life scores shows otherwise. In every group, people used the whole scale, from 0 to 10, and between a third and a half rated their interference 7 or higher: 34% for panic disorder, 46% for depression, and 50% for agoraphobia. For many people, these disorders were not a mild inconvenience.

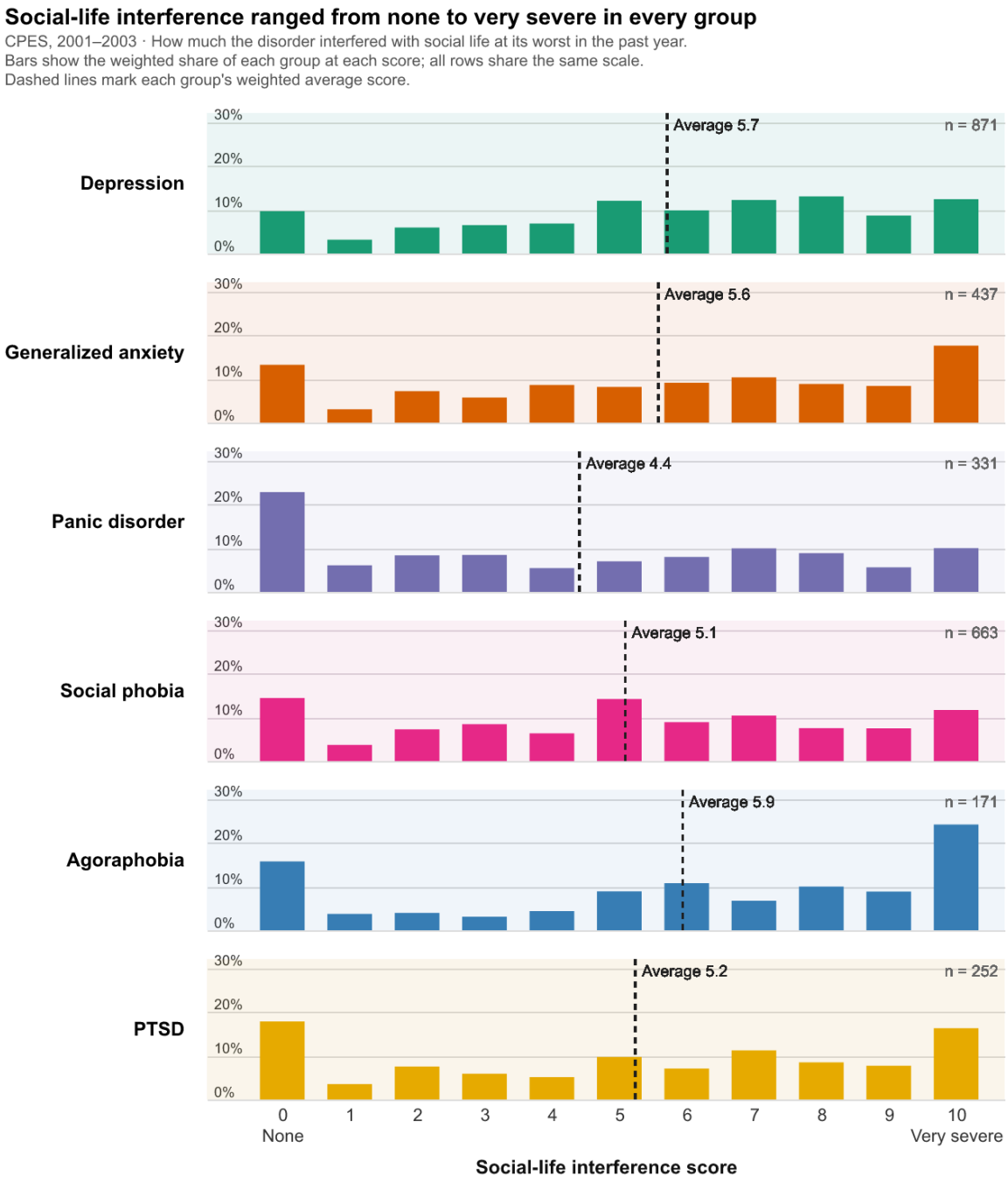


Figure 5. Distribution of social-life interference scores. Dashed lines mark each group's average.

Who got help

Despite this, most people did not report professional care for their disorder in the past year. For five disorders, between 34% and 42% did. Social phobia stands out again: only 18% reported professional care, less than half the rate for depression (39%), even though the two were about equally common.

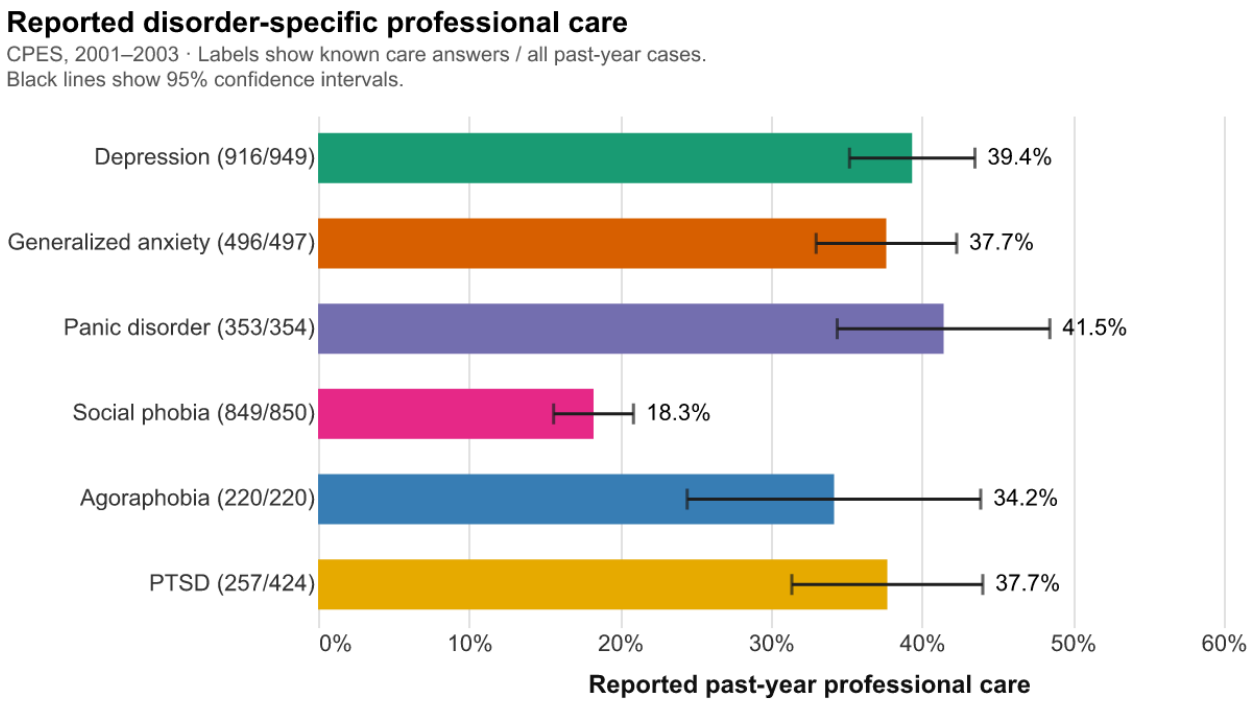


Figure 6. Share reporting past-year professional care for the disorder, among those with a known answer. Black lines show 95% confidence intervals.

Care was more common when interference was worse. In every disorder, the share reporting professional help was higher among people scoring 7–10 than among those scoring 0–3. But even in that highest group, only 33% to 59% reported professional help for their disorder. For social phobia, professional help rose from 12% to 33%, while antidepressant use, which may have been for another condition, reached 47%. Antidepressant use is the one medication question asked of everyone, so it gives a broader view of treatment alongside disorder-specific help.

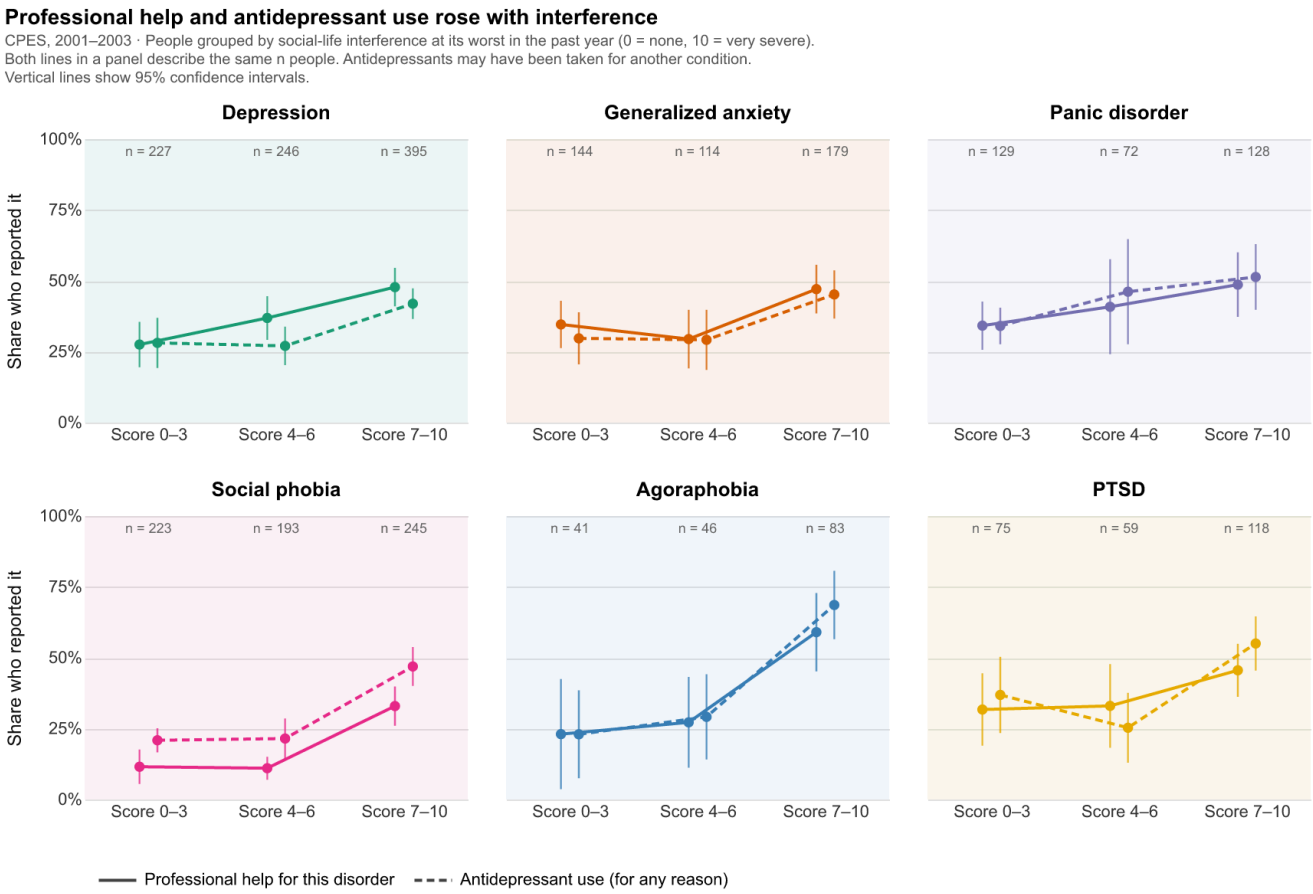


Figure 7. Reported care by social-life interference group. Both lines in a panel describe the same people. Vertical lines show 95% confidence intervals.

Twenty years later

The survey above is more than twenty years old. The 2024 National Survey on Drug Use and Health asks about depression, impairment, and care together, so it offers a recent look at the largest of these groups.

Among adults with past-year depression and severe impairment in daily roles, half reported both professional contact and prescription medication (50.4%). But 29.5% reported neither. Among those without severe impairment, 41.2% reported neither.

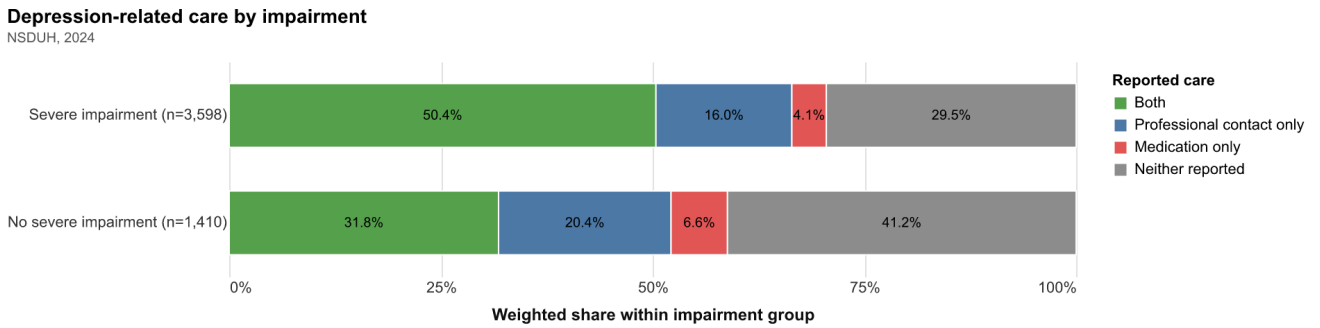


Figure 8. Combinations of professional contact and prescription medication for depression in 2024.

The gap was much wider for adults without health insurance. Among adults with depression and severe impairment, 61.1% of the uninsured reported neither form of care, compared with 25.9% of the insured. This comparison describes two groups; it does not show that insurance caused the difference, since uninsured adults may differ in other ways too.

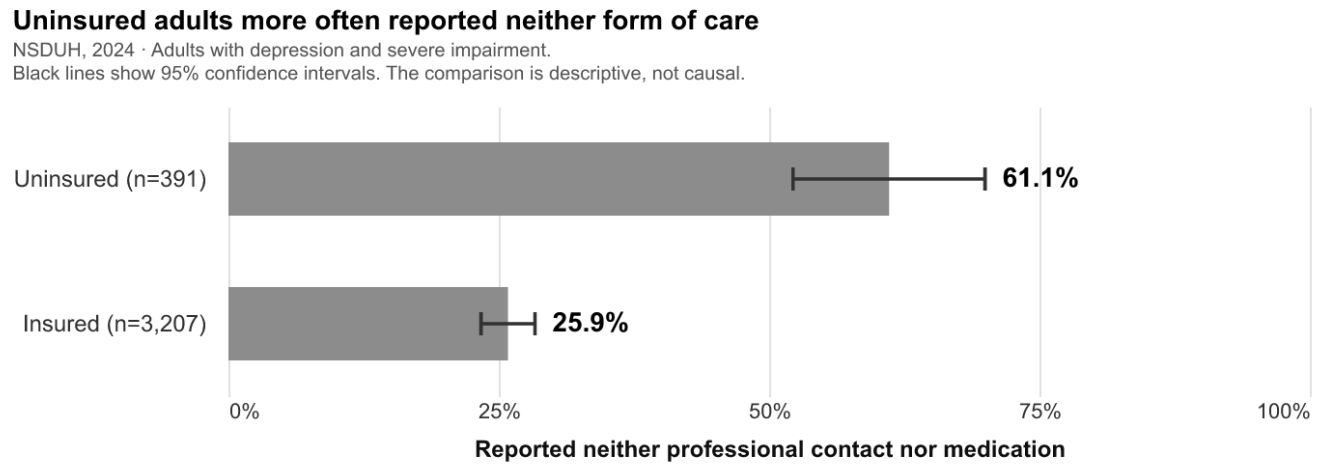


Figure 9. Share reporting neither professional contact nor medication, by insurance. Black lines show 95% confidence intervals.

Putting impairment, insurance, and care together shows how the groups connect. Most adults with depression had severe impairment (68.6%) and insurance (89.5%), and two-thirds reported some form of care (66.8%). The uninsured band is narrow, but most of it flows toward "neither reported": 65.8% of uninsured adults with depression reported no care, compared with 29.3% of insured adults.

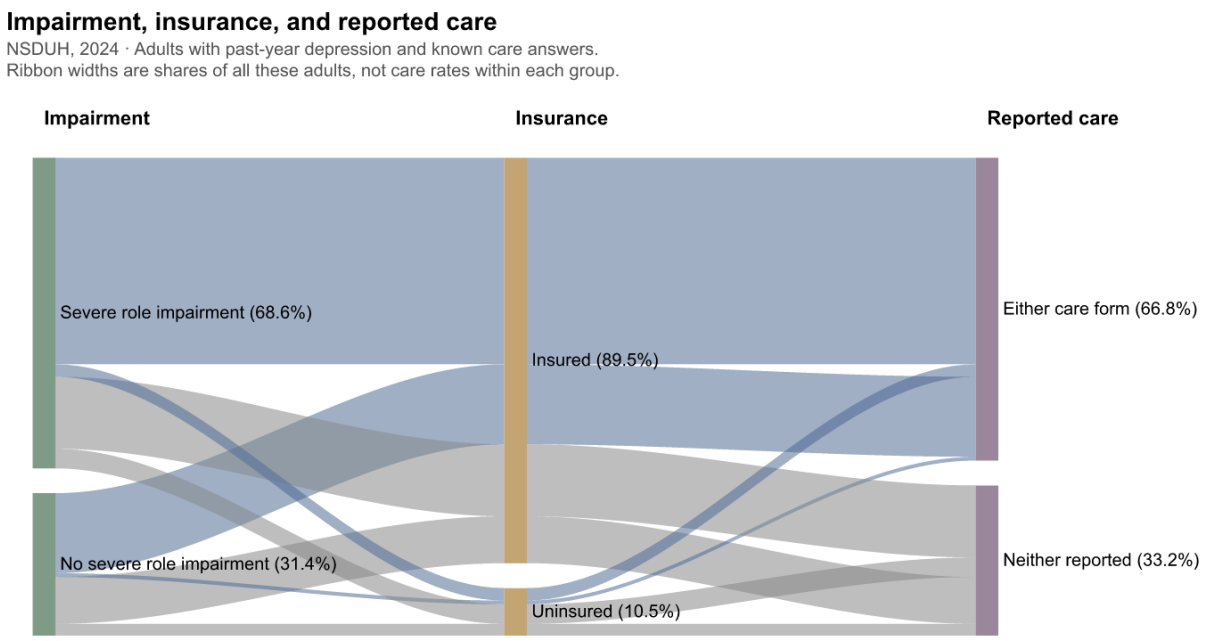


Figure 10. Ribbon widths are shares of all adults with past-year depression and known care answers, not care rates within each group.

Conclusion

Across six common disorders, many people lived with serious disruption to their daily lives, especially their social lives, and most did not report professional care for it. Social phobia shows the gap most clearly: about as common as depression, frequently paired with other disorders, and the least likely to be treated. More severe interference was linked to more care, but even at the highest scores, many people did not report professional help. Two decades later, about one in three adults with depression still reported neither professional contact nor medication, and the share was much higher among the uninsured.

These are descriptive results from surveys, and the two surveys use different questions, so they cannot be compared directly. They do point to the same pattern: the people who are struggling are not always the people who get help.

About the data

Figures 1–7 use the Collaborative Psychiatric Epidemiology Surveys (CPES), restricted to the 10,341 respondents in the NCS-R Part 2 and NLAAS samples. Figures 8–10 use the 2024 National Survey on Drug Use and Health (NSDUH), focusing on 5,008 adults with past-year depression and known answers about care. All percentages and averages use survey weights; confidence intervals account for the survey design. Questions about PTSD care and interference were asked of a smaller, event-specific

group, so the PTSD care estimate uses 257 of 424 cases. Medication questions do not say which condition a medication was for.

Data sources

- Alegría, M., Jackson, J. S., Kessler, R. C., & Takeuchi, D. (2007). *Collaborative Psychiatric Epidemiology Surveys (CPES), 2001–2003 [United States]* (Version 9) [Data set]. Inter-university Consortium for Political and Social Research. https://doi.org/10.3886/ICPSR20240.v9
- Center for Behavioral Health Statistics and Quality. (2025). *2024 National Survey on Drug Use and Health (NSDUH-2024-DS0001)* [Public use data file]. Substance Abuse and Mental Health Services Administration. https://www.samhsa.gov/data/data-we-collect/nsduh-national-survey-drug-use-and-health/datafiles/2024